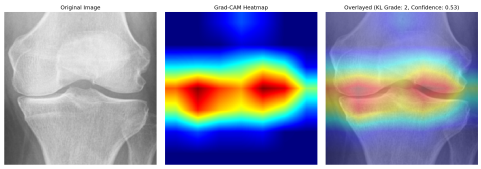
KNEE RADIOLOGICAL REPORT

Report Date:	November 25, 2025	Report ID:	KOA-20251125-49
Patient ID:	test	Age/Sex:	N/A/N/A
DIAGNOSIS:			

Findings:

1. DISCLAIMER: This report was generated with the assistance of artificial intelligence. While AI enhances diagnostic accuracy, the final interpretation and clinical correlation remain the responsibility of the attending radiologist. This report should be interpreted in conjunction with the patient's clinical history and physical examination. **2. CLINICAL HISTORY:** The patient presents with chronic knee pain, stiffness, and limited range of motion, particularly with weight-bearing activities. The indication for this examination is to evaluate for osteoarthritis and other potential causes of knee pain. **3. TECHNIQUE:** Anteroposterior (AP) weight-bearing, lateral, and skyline (patellar) radiographic views of the right knee were obtained. The images were analyzed using an AI-assisted diagnostic tool for osteoarthritis detection and grading. AI Classification Confidence: 83.0%. Kellgren-Lawrence Grading System Applied.



Impression:

Moderate osteoarthritis of the right knee, corresponding to Kellgren-Lawrence Grade 3. The findings indicate significant degenerative changes with moderate joint space narrowing, osteophyte formation, and subchondral bone reaction.

Recommendations:

Recommend conservative management including physical therapy, weight management, and activity modification to reduce stress on the knee joint. Consider further evaluation with MRI if symptoms persist or worsen despite conservative measures to assess for meniscal tears or ligamentous injuries.

Risk Profile:

Patients with Kellgren-Lawrence Grade 3 osteoarthritis are at risk of progressive joint damage and functional decline. Factors influencing prognosis include age, body weight, activity level, and presence of comorbidities. Without appropriate management, the condition may progress to severe osteoarthritis, potentially requiring joint replacement surgery. Long-term outlook depends on adherence to treatment recommendations and lifestyle modifications.

Essential Precautions:

• Daily Activity Modifications: Avoid high-impact activities such as running, jumping, and prolonged standing. Opt for low-impact exercises like swimming or cycling.

• Weight Management: Maintain a healthy weight to reduce stress on the knee joint.

• Proper Footwear: Wear supportive shoes with good shock absorption.

• Monitor for Warning Signs: Be alert for increased pain, swelling, stiffness, or locking of the knee.

• Seek Care When: Consult a healthcare professional if symptoms worsen, interfere with daily activities, or do not respond to conservative management.

Treatment Options:

• Conservative Management: Includes weight loss, activity modification, and the use of assistive devices such as braces or canes.

• Medications: Over-the-counter pain relievers (acetaminophen, NSAIDs) can help manage pain. Prescription NSAIDs or other pain medications may be considered under medical supervision.

• Injections: Corticosteroid injections can provide temporary pain relief. Hyaluronic acid injections (viscosupplementation) may improve joint lubrication and reduce pain.

• Physical Therapy: A physical therapy program can improve strength, flexibility, and range of motion. It can also teach strategies for pain management and joint protection.

• Orthotics: Custom or over-the-counter orthotics can help improve alignment and reduce stress on the knee.

• Surgical Options: In cases of severe pain and functional limitations despite conservative management, surgical options such as arthroscopy or total knee arthroplasty may be considered.

Follow-Up And Monitoring:

Follow-up with an orthopedic specialist or primary care physician is recommended to discuss treatment options and monitor disease progression. Repeat radiographic imaging should be performed periodically (every 1-2 years, or as clinically indicated) to assess for changes in joint space narrowing and osteophyte formation. Monitoring should include assessment of pain levels, functional status, and range of motion. The goal of management is to alleviate pain, improve function, and slow the progression of osteoarthritis.